



EVERGREEN COMMUNITY CARE INC.

MEDICATION ADMINISTRATION

MANUAL

Policy, Procedures, and Practice Standards for Safe Medication Management

All Residential Care Programs — Children and Youth

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Effective Date	July 1, 2026
Review Date	July 1, 2027 (annual); also upon any licensing directive, serious medication incident, or BC regulatory change
Applies To	All staff authorized to administer medications at Evergreen Community Care residential care programs; all Program Coordinators and Program Managers
Legislative Authority	Community Care and Assisted Living Act (CCALA); Residential Care Regulation (RCR) s.45–49; Controlled Drugs and Substances Act (CDSA); BC Pharmacy Operations and Drug Scheduling Act; BC Medication Administration Learning Module (MALM) curriculum; MCFD Standards for Staffed Children's Residential Services (SHSS) Standard G.3
MCFD Standards	Standard C.1 (personal safety); Standard D.3 (access to health services); Standard G.3 (staff training — medication administration certification required before any medication duty)
Related Documents	RC Program Manual v4 (ECC-RC-POL-2026-003); Behaviour Support Manual (ECC-BSP-2026-001); OHS Program Binder (ECC-OHS-POL-2026-002); HR Policy Binder v3 (ECC-HR-POL-2026-001); PC Manual (ECC-PC-MAN-2026-001); PM Manual (ECC-PM-MAN-2026-001)



⚠ This manual governs medication administration at all Evergreen Community Care residential care programs. No staff member administers any medication — prescribed or over-the-counter — until they have completed the required Medication Administration training and been authorized in writing by the Program Manager. Authorization is program-specific and must be renewed annually.



TABLE OF CONTENTS

PART 1 — Policy Foundation

- Section 1 — Policy Statement and Governing Philosophy
- Section 2 — Legislative and Regulatory Framework
- Section 3 — Roles and Responsibilities
- Section 4 — Staff Authorization and Training Requirements

PART 2 — Medication Consent and Documentation

- Section 5 — Consent for Medication Administration
- Section 6 — Medication Administration Record (MAR)
- Section 7 — Individual Medication Profiles

PART 3 — Receiving, Storage, and Security

- Section 8 — Receiving Medications from Pharmacy or Caregiver
- Section 9 — Medication Storage Requirements
- Section 10 — Controlled Drug and Substance Security
- Section 11 — Medication Counts and Reconciliation

PART 4 — Administration Procedures

- Section 12 — The Seven Rights of Medication Administration
- Section 13 — General Administration Procedure — Step by Step
- Section 14 — Oral Medications
- Section 15 — Topical and Transdermal Medications
- Section 16 — Inhaled Medications (Inhalers and Nebulizers)
- Section 17 — Eye and Ear Drops
- Section 18 — As-Needed (PRN) Medications
- Section 19 — Over-the-Counter (OTC) Medications
- Section 20 — Youth Self-Administration

PART 5 — Special Medication Categories

- Section 21 — Controlled Drugs and Substances (CDS)
- Section 22 — Psychotropic and Psychiatric Medications
- Section 23 — Emergency Medications and Auto-Injectors (EpiPen)
- Section 24 — Medications Requiring Special Monitoring

PART 6 — Medication Refusal and Non-Administration

- Section 25 — Youth Refusal of Medication
- Section 26 — Missed or Late Doses
- Section 27 — Medication Withheld by Clinical Direction

PART 7 — Medication Errors

- Section 28 — Definition and Classification of Medication Errors
- Section 29 — Medication Error Response Procedure
- Section 30 — Medication Error Reporting and Documentation
- Section 31 — Root Cause Analysis and Prevention

PART 8 — Medication Review and Disposal

- Section 32 — Medication Review and Reconciliation
- Section 33 — Expiry, Discontinuation, and Disposal



Section 34 — Transfer and Discharge Medication Procedures

PART 9 — Health Monitoring and Emergencies

Section 35 — Monitoring Medication Effects and Side Effects

Section 36 — Adverse Drug Reactions

Section 37 — Medical Emergencies Related to Medication

PART 10 — Quality Assurance and Oversight

Section 38 — MAR Audit and Quality Review

Section 39 — QA Indicators — Medication Safety

Section 40 — Pharmacy and Prescriber Relationships

PART 11 — Appendices

Appendix A — Staff Medication Authorization Form

Appendix B — Medication Administration Record (MAR) Template

Appendix C — Controlled Drug and Substance Count Sheet

Appendix D — Medication Error Report Form

Appendix E — Medication Incident — Poison Control and Emergency Reference

Appendix F — PRN Medication Administration and Effectiveness Record

Appendix G — Medication Disposal Record

Appendix H — Youth Medication Refusal Record

Appendix I — Medication Administration Staff Acknowledgement



PART 1 — Policy Foundation

1. POLICY STATEMENT AND GOVERNING PHILOSOPHY

Evergreen Community Care Inc. is committed to the safe, accurate, and dignified administration of medications to every child and youth in its care. Medication administration is a clinical act that carries significant responsibility. When done correctly, it supports health and wellbeing. When done incorrectly, it can cause serious harm. This manual establishes the minimum standards, procedures, and safeguards that apply to every medication-related act at every Evergreen Community Care residential care program.

No shortcuts. No exceptions. Every dose matters.

1.1 Governing Principles

Principle	What It Means in Practice
Safety first — always	Every procedure in this manual exists to prevent harm. If you are ever uncertain about a medication, a dose, or a procedure: STOP. Verify before administering. Never guess.
Authorization required	No staff member administers any medication without written program-specific authorization from the Program Manager. Authorization is based on completed training and demonstrated competency.
The Seven Rights	Every medication administration follows the Seven Rights without exception: Right Youth, Right Medication, Right Dose, Right Route, Right Time, Right Reason, Right Documentation.
Youth rights and dignity	Medications are administered with full respect for the youth's dignity. Youth are informed about their medications. Refusal is handled therapeutically. Medications are never administered covertly or as a restraint or behaviour management tool.
Accurate documentation	If it isn't documented, it didn't happen. Every dose administered or not administered is documented on the MAR on the same shift. MAR entries are never altered.
Prescriber authority	Medications are administered exactly as prescribed. Staff do not alter doses, skip doses, or modify routes of administration without prescriber direction. Any concern about a prescription is raised with the prescriber directly.
Transparent error reporting	Medication errors are reported immediately, honestly, and completely. The error reporting system exists to improve safety — not to punish. Fear of reporting is more dangerous than the error itself.
Controlled substance	Controlled drugs and substances are counted, secured, and



integrity	documented with absolute precision at every shift change. Any discrepancy is investigated the same shift.
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2. LEGISLATIVE AND REGULATORY FRAMEWORK

Authority	Relevance	Key Requirements
Residential Care Regulation (RCR), BC Reg. 96/2009, amended April 2026	Primary BC regulation governing medication administration in residential care facilities	s.45 — medications stored safely; s.46 — medications administered only by authorized persons; s.47 — medication records required; s.48 — controlled substances double-locked; s.49 — disposal requirements
Community Care and Assisted Living Act (CCALA)	Enabling legislation for residential care facilities; licensing authority	Licensing conditions may specify additional medication administration requirements; Licensing Officer has authority to inspect medication records and storage at any time
Controlled Drugs and Substances Act (CDSA), federal	Governs all Schedule I through VIII controlled substances including narcotics, benzodiazepines, stimulants, and other controlled substances	Strict possession, administration, and disposal requirements; discrepancies may constitute a criminal offence; police involvement may be required for unexplained losses
BC Pharmacy Operations and Drug Scheduling Act and Regulations	Governs drug scheduling and dispensing in BC	Prescription drugs must be prescribed by a licensed prescriber and dispensed by a licensed pharmacy; original pharmacy labels must be maintained; dispensing changes require pharmacy authorization
MCFD Standards for Staffed Children's Residential Services (SHSS), April 2026	MCFD operational standards for contracted residential care providers	Standard G.3 — medication administration training required before any medication duty; Standard D.3 — youth have the right to access required health services including medications; Standard C.1



		— medication security as a personal safety requirement
BC Medication Administration Learning Module (MALM)	The BC provincial curriculum for non-regulated staff medication administration in residential care	Staff must complete MALM or equivalent approved curriculum before administering medications; annual competency renewal required
Health Professions Act (BC)	Governs regulated health professions including nurses and pharmacists	Nurses and pharmacists may administer medications without separate authorization under their professional registration; all other staff require MALM or equivalent and program-specific authorization

3. ROLES AND RESPONSIBILITIES

Role	Medication Administration Responsibilities
Youth Care Worker (YCW) — authorized only	Administer medications exactly as prescribed and documented on the MAR; complete the Seven Rights verification before every dose; document every administration or non-administration on the MAR before leaving the medication area; report any error, near-miss, or concern immediately; never administer any medication without current authorization; conduct controlled substance counts at every shift change
Shift Lead	All YCW responsibilities plus: verify the medication cabinet is locked at every shift; supervise controlled substance counts at shift change; ensure all MAR entries for the shift are complete before outgoing shift departs; respond to and escalate any medication concern immediately
Program Coordinator (PC)	Receive and action all new medication orders within 24 hours; review MAR weekly for completeness, accuracy, and anomalies; maintain the medication storage area in good order; maintain the controlled substance count sheet; report all medication errors to PM within 2 hours; complete monthly MAR audit; ensure medication cabinet keys are accounted for at all times; maintain medication authorization records for all staff
Program Manager (PM)	Issue and renew medication administration authorization for all eligible staff; review all medication error reports within 24 hours; co-sign all medication error reports; review PC monthly MAR audit; establish and maintain the relationship with the program pharmacy and prescribers; review the controlled substance register monthly; approve all PRN medication protocols; notify MCFD of any medication error constituting



	a Category 1 or 2 CI; ensure all authorized staff have current training certification
Executive Director (ED)	Approve this manual annually; receive PM quarterly medication incident trend report; ensure programs are resourced for safe medication administration; respond to any licensing directive related to medication management
Prescribing Physician / Nurse Practitioner	Prescribe medications in writing using standard prescription format; respond to medication questions from program staff within clinically appropriate timeframes; provide clear instructions for PRN medications including target symptom, dose, frequency, maximum dose, and monitoring parameters
Dispensing Pharmacy	Dispense medications in compliance with BC pharmacy legislation; label all medications with required information; provide blister pack or dosette dispensing where appropriate; maintain a medication profile for each youth; advise program staff on medication interactions, storage, and administration questions

4. STAFF AUTHORIZATION AND TRAINING REQUIREMENTS

4.1 Authorization Requirements

No staff member administers any medication — prescribed or over-the-counter — until they hold written program-specific medication administration authorization issued by the Program Manager. Authorization is not transferable between programs.

Requirement	Detail
Training completion	BC Medication Administration Learning Module (MALM) or equivalent approved curriculum completed and documented
Competency demonstration	Practical competency demonstration to PC or PM before authorization issued; staff demonstrate the Seven Rights procedure, MAR documentation, controlled substance count, and error reporting
Written authorization	PM issues written authorization (Appendix A); copy in staff personnel file; copy in medication administration authorization binder at the program
Annual renewal	Authorization expires 12 months after issue; renewal requires: current MALM certificate; annual competency review by PC; PM re-authorization in writing
Program-specific authorization	Authorization is specific to the program where training was completed; staff working at a different program require separate authorization from that program's PM



Suspension of authorization	PM may suspend authorization immediately if: training expires; a serious medication error occurs; competency concerns arise; CRC or employment concerns arise. Suspension takes effect immediately and staff are removed from all medication duties.
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4.2 Training Content Requirements

Training Module	Content	Delivery
Module 1 — Medication Fundamentals	Drug categories; pharmacology basics; prescription reading; drug schedules; medication forms and routes; terminology	MALM online + in-person orientation with PC
Module 2 — The Seven Rights	Right Youth, Right Medication, Right Dose, Right Route, Right Time, Right Reason, Right Documentation; verification procedure; common errors	MALM online + practical demonstration
Module 3 — MAR Documentation	MAR completion; late entries; corrections; controlled substance count sheet; error documentation	In-person with PC using actual MAR format
Module 4 — Controlled Drugs and Substances	CDS categories; double-lock requirement; count procedure; discrepancy response; CDSA obligations; documentation	In-person with PM or PC; demonstration of count procedure
Module 5 — Medication Storage and Security	Temperature requirements; light and humidity; cabinet and refrigerator requirements; key management; disposal	Facility walkthrough with PC
Module 6 — Medication Errors	Error classification; immediate response; reporting chain; documentation; near-miss reporting; root cause analysis	MALM online + case-based discussion with PC
Module 7 — Special Medications	PRN protocols; psychotropic medications; EpiPen and emergency medications; inhalers and nebulizers; topical and transdermal	In-person with PC; return demonstration for each route
Module 8 — Youth Rights and Refusal	Youth's right to medication information; refusal procedure; never covert administration; dignity in administration	In-person discussion with PC
Annual Competency Review	Practical review of: Seven Rights; MAR documentation; CDS count; error scenario response	Annual in-person review with PC or PM before re-authorization

PART 2 — Medication Consent and Documentation

5. CONSENT FOR MEDICATION ADMINISTRATION

Medications are administered to youth in care only when appropriate consent has been obtained and documented. The nature of consent depends on the youth's age, capacity, and legal status.

Consent Situation	Required Process
Prescribed medication — youth under 19 in MCFD care	MCFD social worker provides written consent or authorization as part of the placement documentation; ongoing prescriptions do not require re-consent at every administration unless there is a material change
New prescription initiated during placement	Prescribing physician obtains informed consent from the authorized consent provider (parent, guardian, or MCFD as applicable per CFCSA); PC documents the new prescription, authorization, and consent status in the youth's medication record before first administration
Over-the-counter medication	PC obtains verbal or written confirmation from MCFD social worker before administering any OTC medication not previously authorized; documented on the MAR with the consent source noted
Youth 16 or over with capacity to consent	Under BC's Infants Act, a youth 16 or over who has capacity may consent to their own health care; in practice for residential care, the PC consults with the MCFD social worker and prescriber on any consent questions involving youth approaching majority
Emergency medication	When a youth requires emergency medication and there is no time to obtain consent: 911 is called; paramedics manage emergency medication; the emergency department manages clinical consent; PC notifies MCFD social worker immediately
Consent withdrawal or change	Any change to consent status is communicated to the PC and PM immediately; PC contacts the prescriber and MCFD social worker; no medication is administered or withheld based on disputed consent without PM and MCFD direction

6. MEDICATION ADMINISTRATION RECORD (MAR)

The Medication Administration Record (MAR) is the legal document of all medications administered or not administered at the program. It is the primary tool for ensuring medication safety, continuity of care, and regulatory compliance. The MAR is a legal document — it must be accurate, complete, and unaltered.



6.1 MAR Standards

Standard	Requirement
Format	The MAR format used at Evergreen Community Care is the template at Appendix B, or the pharmacy-generated MAR where the program uses a blister pack or pharmacy-generated dispensing system. Both formats are acceptable.
One MAR per youth per month	A new MAR is initiated on the first day of each calendar month for each youth with active medications; historical MARs are retained in the youth's medication file for the required retention period
One line per medication	Each prescribed medication occupies its own line; OTC medications authorized for this youth are included; PRN medications are on a separate line with PRN notation
Completion timing	The MAR is completed at the time of administration or non-administration — not at the end of the shift. Pre-signing the MAR for medications not yet administered is absolutely prohibited.
Ink only	All MAR entries are made in black or blue ink; pencil is prohibited; correction fluid (white-out) is prohibited
Error correction	Errors on the MAR are corrected by drawing a single line through the incorrect entry, initialling it, dating it, and entering the correct information. Never obscure an entry.
Legibility	All entries must be legible. If another staff member cannot read an entry, it is not useful. Print clearly.
Retention	MARs are retained in the youth's permanent medication record for the duration of care plus 7 years minimum (10 years where the youth had any controlled substance medications)

6.2 MAR Entry Requirements

For every scheduled medication, for every administration cycle, the MAR must show one of the following:

Entry Code	Meaning and Documentation Required
Staff initials	Medication administered as prescribed. Full initials in the scheduled time column. Initials are expanded (full name and initials) in the signature legend at the bottom of each MAR page.
R — Refused	Youth refused the medication. Circle 'R' in the time column. Document the refusal in the shift log: time; what was offered; how the youth refused; what was done; outcome. Complete the Medication Refusal



	Record (Appendix H) for any refusal.
H — Held	Medication held on clinical direction from the prescriber. Circle 'H'. Document the hold in the shift log: name of prescriber; instruction given; time; date. This is not a missed dose — it is a clinically directed hold.
NA — Not Available	Medication not available — not yet dispensed by pharmacy or ran out. Circle 'NA'. Notify PC immediately. Document steps taken to obtain the medication. Never leave a scheduled dose unaccounted for.
S — Self-Administered	Youth self-administered the medication under staff supervision (where self-administration is authorized in the medication profile). Circle 'S'. Staff must observe the self-administration; self-administration without staff observation is not authorized.
T — Transcription Error	Used only when a scheduled time was entered incorrectly by the prescriber or on the MAR. Requires PC co-signature and written explanation. Consult PC before using this code.

7. INDIVIDUAL MEDICATION PROFILES

Every youth with active medications has an Individual Medication Profile maintained by the PC. The medication profile is the comprehensive record of all medications for this youth and is reviewed at every admission, at every prescription change, and monthly.

Medication Profile Component	Required Content
Youth identification	Legal name; date of birth; allergies and adverse reactions — prominently displayed at the top of every page
Known allergies and reactions	All known drug allergies; type of reaction (anaphylaxis, rash, GI upset); medications to avoid; NKDA (No Known Drug Allergies) noted explicitly if that is the case
Current medications — prescribed	Each medication: generic and brand name; prescriber; indication; dose; route; frequency; start date; renewal date; any monitoring requirements
Current medications — OTC authorized	Each authorized OTC medication: indication; maximum dose; frequency; authorization source and date
Discontinued medications	Last 12 months of discontinued medications retained; reason for discontinuation noted
PRN medications	Each PRN medication: target symptom; dose; route; maximum frequency; monitoring requirements; effectiveness tracking requirement



Known drug interactions	Any identified drug-drug interactions documented; clinical significance noted; prescriber acknowledgement that the interaction is being actively managed
Special instructions	Any medication-specific special instructions: timing relative to food; specific administration technique; monitoring parameters; signs of toxicity
Medication history at admission	Medications on admission documented in full; any medications not continued and reason noted
Consent and authorization record	Consent source for each medication; authorization date; review dates



PART 3 — Receiving, Storage, and Security

8. RECEIVING MEDICATIONS FROM PHARMACY OR CAREGIVER

Step	Action	Documentation
1. Verify identity	Confirm the medication is for the correct youth: name on label matches the youth's legal name; date of birth matches if included	Check against the Individual Medication Profile
2. Verify the prescription label	Confirm: prescriber name; medication name (generic and/or brand); dose; route; frequency; dispensing date; expiry date; pharmacy name and contact	Any discrepancy between the label and the prescription: call the pharmacy immediately; do not administer
3. Check quantity	Count the tablets, capsules, or measure liquid volume; confirm it matches the dispensed quantity on the label	Document received quantity on the MAR and controlled substance count sheet (if CDS)
4. Check condition	Confirm all tablets are intact; no unusual colour, odour, or appearance; packaging is intact and not tampered with	Any concern about medication condition: do not administer; contact pharmacy; document
5. Update the medication profile	If this is a new medication or a renewal: update the Individual Medication Profile; update the MAR for the current month	PC updates medication profile; PM reviews and signs new medications
6. Store immediately	Store the medication in the locked medication cabinet (or double-locked cabinet for CDS) immediately upon receipt	Do not leave medications unattended outside locked storage at any time
7. Medications from caregivers at admission	Any medication brought by a family member or caregiver at admission: record on the medication profile; verify against current prescriptions; contact the prescriber or pharmacy if uncertain; do not administer until verified	PC documents all medications at admission; PM reviews within 24 hours

9. MEDICATION STORAGE REQUIREMENTS

Storage Requirement	Standard
Primary medication cabinet	Locked at all times except during authorized administration; one key per shift to a designated authorized staff member; cabinet is solid,



	secured to wall or floor, and inaccessible to youth
Temperature	Room temperature medications stored at 15–25°C; check cabinet temperature monthly; thermometer in every medication cabinet; document temperature on monthly inspection checklist
Refrigerated medications	Stored in a dedicated locked medication refrigerator (or a clearly labelled locked section of the facility refrigerator inaccessible to youth); temperature maintained at 2–8°C; thermometer with min/max reading; temperature documented daily when refrigerated medications are in use
Light-sensitive medications	Stored in original amber containers or opaque packaging as dispensed; never transferred to clear containers
Medication-specific requirements	Any storage requirement noted on the pharmacy label is followed exactly (e.g., 'protect from light', 'keep upright', 'do not freeze'); specific requirements documented in the Individual Medication Profile
Separation of medications	Each youth's medications stored separately and clearly labelled; medications from different youth never mixed in the same container or storage area
Original containers	Medications are stored in the original pharmacy-dispensed container with the original pharmacy label intact; never transferred to unmarked containers
Cabinet inventory	PC conducts a monthly medication inventory: counts all medications; confirms expiry dates; removes expired medications; reconciles against the MAR and medication profiles
Key management	Medication cabinet keys are accounted for at every shift change; a key log is maintained; lost keys are reported to PC immediately; PM is notified of any key loss; locks are changed if a key is unaccounted for

10. CONTROLLED DRUG AND SUBSTANCE SECURITY

Controlled drugs and substances (CDS) — Schedule I through IV medications including narcotics, benzodiazepines, stimulants, and other CDSA-scheduled substances — require additional security measures beyond those for regular medications.

CDS Security Requirement	Standard
Double-locked storage	CDS are stored in a locked container within the locked medication cabinet — a locked box within a locked cabinet. Both locks must be engaged at all times. Single-lock storage of CDS is never acceptable.
CDS key management	The CDS inner lock key is kept separately from the outer cabinet key;



	CDS key is held by the senior authorized staff member on duty; CDS key is transferred at every shift change with the count; key loss is reported immediately to PC and PM
Minimum two-person count	Every CDS count requires two authorized staff members — one counting and one witnessing; both sign the count sheet; counts occur at every shift change without exception
Count procedure	Count every CDS item individually at each shift change: physical count of tablets/capsules/mL; compare to the count sheet; explain any discrepancy before both staff sign; if discrepancy cannot be explained: see Section 11
Visitor access	No visitor, family member, student, or volunteer has access to the medication cabinet or CDS storage at any time; this rule has no exceptions

11. MEDICATION COUNTS AND RECONCILIATION

11.1 Scheduled Count Procedure — All Medications

- Outgoing and incoming shift lead perform a medication count at every shift change
- Count all scheduled medications: confirm quantities match the MAR record of doses administered
- Count all CDS with two authorized staff: count matches the CDS Count Sheet (Appendix C)
- Confirm medication cabinet is locked; CDS storage is double-locked; keys are transferred
- Both outgoing and incoming shift lead sign the count record before the outgoing shift departs

11.2 Discrepancy Response

Discrepancy Type	Required Response
Count discrepancy — more medication than expected (unexplained)	Do not assume error; secure the medication; notify PC immediately; PC notifies PM same shift; investigate before the outgoing shift leaves; document all findings and steps taken
Count discrepancy — less medication than expected (potential loss)	Immediately notify PC; PC notifies PM; PM investigates same shift; review MAR for undocumented administrations; review all staff who had key access since the last count; if loss cannot be explained, PM notifies ED, licensing, and police (CDSA requirement for unexplained CDS loss); full written incident report within 24 hours
Discrepancy explained by documentation error	Correct the documentation; explain fully in the count sheet and shift log; both counting staff sign the explanation; PM reviews and co-signs;



	incident report if any policy deviation occurred
Recurring discrepancies	PM escalates to ED; review staff access and processes; consider whether a criminal investigation is warranted; notify MCFD if pattern suggests a safety concern for youth



PART 4 — Administration Procedures

12. THE SEVEN RIGHTS OF MEDICATION ADMINISTRATION

The Seven Rights are verified before every medication administration — every dose, every time, without exception. Skipping a Right is never justified by time pressure, familiarity with the youth, or routine. Most medication errors occur because a Right was not verified.

Right	Verification Procedure
1. RIGHT YOUTH	Verify the youth's identity by name before preparing any medication. Use two identifiers where possible: full name and date of birth. Do not rely on room number or appearance. Ask the youth to state their name. If there is any doubt: stop and verify.
2. RIGHT MEDICATION	Read the medication label three times: when taking the medication from storage; when preparing the dose; before administering. The label must match the MAR exactly — generic name, brand name, or both. If in doubt: stop and verify with the pharmacy or prescriber.
3. RIGHT DOSE	Calculate and confirm the dose before preparing. If the prescribed dose requires calculation (e.g., mg/kg): show your calculation to a second staff member. If a dose seems unusually large or small: stop and call the pharmacy or prescriber before administering.
4. RIGHT ROUTE	Confirm the route of administration matches the prescription label and the MAR. Oral, topical, inhaled, eye, and ear are all different and are never interchanged. Never administer a medication by a route not specified on the prescription.
5. RIGHT TIME	Administer within 30 minutes before or after the scheduled time. If the youth is asleep, at school, or otherwise unavailable: document on the MAR and consult the Individual Medication Profile for specific instructions. Some medications have critical timing requirements — check the profile.
6. RIGHT REASON	Know why this youth takes this medication. If a medication appears on the MAR with no documented indication: ask the PC before administering. Never administer a medication whose purpose is unknown to you.
7. RIGHT DOCUMENTATION	Document the administration on the MAR immediately after giving the medication — not before, not at the end of the shift. Document: your initials; the actual time administered; any relevant observations. A dose is not documented until it has been given.



⚠ Pre-signing the MAR before administering a medication is a serious medication administration error. It is prohibited under any circumstances. If the youth refuses or the dose cannot be administered for any reason, the pre-signed entry becomes a false record.

13. GENERAL ADMINISTRATION PROCEDURE — STEP BY STEP

Step	Action	Critical Points
1. Prepare	Wash hands thoroughly with soap and water for at least 20 seconds, or use alcohol-based hand sanitizer. Gather required supplies: medication; measuring device if liquid; MAR; gloves if indicated.	Hand hygiene is mandatory before every medication administration. Gloves are required when handling topical medications or when there is any risk of exposure to bodily fluids.
2. Retrieve from storage	Unlock the medication cabinet. Take only the medications for the current administration. Read the label as you remove it from storage — Right Medication, first check.	Lock the cabinet immediately if you are interrupted. Never leave the cabinet unlocked and unattended.
3. Verify the Seven Rights	Complete all Seven Rights verifications before preparing the dose. Check the label against the MAR. Calculate the dose if needed.	If any Right cannot be confirmed: do not administer. Return the medication to storage and notify the PC immediately.
4. Prepare the dose	Prepare the dose according to the medication type (see Sections 14–19). Handle tablets and capsules with a medication cup — do not touch with bare hands. Measure liquids in a calibrated oral syringe or medicine cup.	Never crush, cut, or open capsules without pharmacy confirmation that this is appropriate for the specific medication. Some medications must not be altered.
5. Identify the youth	Confirm the youth's identity using two identifiers: name and date of birth. Ask the youth to state their full name. Never assume identity from appearance or room number.	If the youth is unable or unwilling to confirm identity: do not administer. Notify the PC.
6. Administer the medication	Administer the medication to the youth using the correct technique for the route. Observe the youth taking the medication — confirm it has been swallowed or applied.	Stay with the youth until the medication has been taken. For oral medications: observe swallowing. If you suspect the medication was not swallowed (cheeked): do not attempt to retrieve it; notify the PC; document.
7. Document immediately	Enter your initials and the actual time administered on the MAR immediately after	The MAR entry is the final step of every administration.



	administration — while still at the medication preparation area.	It is not optional. It is not done later.
8. Return and secure	Return the medication container to the correct storage location. Lock the medication cabinet. Wash hands.	Confirm the cabinet is locked before leaving the medication area.
9. Observe and monitor	Observe the youth for any immediate adverse reaction following administration. For medications with known side effects: conduct monitoring per the Individual Medication Profile and document observations in the shift log.	Any adverse reaction: stay with the youth; call the PC; call 911 if the reaction is severe; see Section 36.

14. ORAL MEDICATIONS

14.1 Tablets and Capsules

- Use a medication cup or spoon to transfer tablets and capsules from the container — do not handle with bare hands
- Offer water with oral medications unless contraindicated
- Observe the youth swallowing — remain present until swallowing is confirmed
- If the youth cannot swallow tablets: consult the pharmacy before crushing or splitting; some medications cannot be altered; crushing or splitting without pharmacy authorization is a medication error
- Enteric-coated, extended-release, and sublingual medications are never crushed or split without explicit pharmacy authorization

14.2 Liquid Medications

- Use a calibrated oral syringe or graduated medicine cup — never a household spoon, which is not calibrated
- Shake suspensions thoroughly before measuring unless contraindicated on the label
- Measure at eye level to read the correct meniscus
- Oral syringes used for oral medications are never used for any other purpose and are labelled 'ORAL ONLY'

15. TOPICAL AND TRANSDERMAL MEDICATIONS

- Wear gloves when applying any topical or transdermal medication to prevent absorption through staff member's skin
- Transdermal patches: note and document the application site; rotate sites as prescribed; remove the previous patch before applying a new one; dispose of used patches safely — fold sticky sides together and place in a sharps or secure waste container



- Topical creams, ointments, and gels: apply the prescribed amount to the prescribed area; use gloved finger or applicator; never apply with bare hands
- Eye and ear preparations are not topical medications — see Section 17

16. INHALED MEDICATIONS (INHALERS AND NEBULIZERS)

16.1 Metered-Dose Inhalers (MDI)

- Shake the inhaler for 5 seconds before use unless the prescriber has specified otherwise
- Use a spacer device where prescribed or where the youth has difficulty coordinating inhalation
- Position: seated upright; exhale fully before actuation; inhale slowly and deeply during actuation; hold breath for 10 seconds
- Wait 1 minute between puffs of the same medication; wait the prescribed time between different inhaled medications
- Clean the spacer weekly per manufacturer instructions; replace per manufacturer schedule

16.2 Nebulizers

- Use sterile or distilled water as the diluent unless otherwise prescribed
- Assemble the nebulizer using clean technique; wash hands before assembly
- Administer over the prescribed time with the youth sitting upright
- Clean the nebulizer mask and cup after every use per manufacturer instructions
- Document the treatment duration and the youth's response

17. EYE AND EAR DROPS

17.1 Eye Drops and Ointments

- Wash hands; wear gloves
- Ask the youth to tilt their head back and look up; gently pull down the lower eyelid to create a pocket
- Instil drops into the pocket — never directly onto the cornea; do not allow the dropper tip to touch the eye or any surface
- Ask the youth to close the eye gently for 1–2 minutes; apply gentle pressure to the inner corner of the eye (nasolacrimal duct occlusion) to reduce systemic absorption for medications where this is indicated
- Ointments: apply a thin ribbon along the inside of the lower eyelid; never apply directly to the eye surface



17.2 Ear Drops

- Warm the drops to body temperature by holding the bottle in your hands for 2 minutes — cold drops can cause dizziness
- Position the youth with the affected ear upward; for youth under 3: pull the ear lobe down and back; for youth 3 and older: pull the outer ear up and back
- Instil the prescribed number of drops; gently press on the small flap (tragus) to help the drops enter the ear canal
- Keep the youth in position for 2–5 minutes; a cotton ball may be placed loosely if prescribed

18. AS-NEEDED (PRN) MEDICATIONS

PRN medications are administered based on a clinical judgment that the target symptom or condition is present. They are not administered on a fixed schedule. Every PRN administration requires both a clinical decision and documentation of the reason.

PRN Requirement	Standard
Written PRN protocol	Every PRN medication must have a written protocol approved by the PM and prescriber (or pharmacist confirmation) specifying: target symptom or condition; dose; route; maximum frequency; maximum dose in 24 hours; monitoring requirements; when to escalate
Clinical assessment before administration	Assess whether the target symptom or condition is present before administering a PRN medication; document the assessment finding in the PRN record and the shift log
PRN documentation	Complete the PRN Medication Administration and Effectiveness Record (Appendix F) for every PRN administration: pre-administration symptom assessment; dose and time administered; post-administration effectiveness assessment; onset of action; any side effects
Frequency monitoring	PC reviews PRN usage weekly; any PRN used more than 3 times in 7 days is flagged for prescriber review; PM is notified; prescriber is consulted about whether the PRN is meeting the clinical need or whether a scheduled medication is warranted
PRN effectiveness	Document whether the PRN was effective: what changed (or did not change) after administration; this data is reviewed at prescriber appointments

19. OVER-THE-COUNTER (OTC) MEDICATIONS

Over-the-counter medications are not automatically authorized. Every OTC medication administered at Evergreen Community Care requires specific authorization before use.



OTC Authorization Requirement	Detail
Prescriber or MCFD authorization required	Before administering any OTC medication, the PC must have written or documented verbal authorization from: the prescribing physician; or the MCFD social worker (for single or emergency use). Standing OTC authorizations are renewed at each annual medication profile review.
Common standing OTC authorizations	Programs typically maintain standing OTC authorizations for: acetaminophen (Tylenol) for pain and fever; ibuprofen (Advil/Motrin) for pain and fever; throat lozenges; saline nasal spray; sunscreen; insect repellent. All require prescriber authorization and documentation.
OTC contraindications	OTC medications are not administered if the youth has a known allergy or contraindication; consult the Individual Medication Profile and pharmacy if any doubt exists
Natural health products and supplements	Vitamins, herbal products, probiotics, and other supplements follow the same authorization requirement as OTC medications; they are not automatically permitted
Dosing	OTC medications are administered at the dose recommended for the youth's weight and age by the prescriber or on the product label. Never exceed the recommended dose.

20. YOUTH SELF-ADMINISTRATION

Some youth, particularly older adolescents, may be authorized to self-administer their own medications as part of their transition to independence and in support of their right to learn life skills. Self-administration requires explicit authorization and does not remove the staff member's oversight obligation.

Self-Administration Requirement	Standard
Authorization	Self-administration is authorized in writing in the youth's Individual Medication Profile; requires: prescriber direction or agreement; PM approval; MCFD social worker awareness
Staff observation requirement	Authorized self-administration is performed in the presence of an authorized staff member who observes the youth taking the medication and confirms it has been consumed or applied correctly
Documentation	Staff document the self-administration on the MAR using the 'S' code; staff never pre-sign a self-administration entry; if the youth does not self-administer as authorized, document as a refusal or hold as applicable



Medication storage for self-administering youth	Even where self-administration is authorized, medications remain in the locked medication cabinet and are retrieved by staff for each self-administration event; youth do not retain personal supplies of medications in their rooms unless specifically authorized in writing by the PM and prescriber for very specific circumstances (e.g., asthma rescue inhaler)
Rescue inhaler exception	A youth may keep their own rescue inhaler (e.g., salbutamol/Ventolin) on their person or in their room if: prescribed for immediate access; documented in the medication profile; youth has demonstrated correct use; PM and prescriber have authorized; staff know the inhaler is in the youth's possession and monitor supply

PART 5 — Special Medication Categories

21. CONTROLLED DRUGS AND SUBSTANCES (CDS)

Controlled drugs and substances require the highest level of care and documentation in medication management. The following procedures apply in addition to all general medication administration requirements.

CDS Schedule	Examples	Additional Requirements
Schedule I — Opioids and Narcotics	Morphine, oxycodone, hydromorphone, codeine products (above threshold), fentanyl	Double-locked storage; count at every shift change with two staff; any loss immediately reported to PM, licensing, and police (CDSA requirement)
Schedule III — Benzodiazepines	Lorazepam, clonazepam, diazepam, alprazolam	Double-locked storage; count at every shift change with two staff; tapering schedules followed exactly — never abruptly stopped
Schedule IV — Stimulants and sedatives	Methylphenidate (Ritalin, Concerta), amphetamines (Adderall, Vyvanse), zopiclone, some barbiturates	Double-locked storage; count at every shift change with two staff; stimulants: document time of administration carefully — timing affects sleep

21.1 CDS Administration Requirements

- Two authorized staff present at every CDS administration: one administers, one witnesses and documents
- Both staff sign the CDS Count Sheet at the time of administration: dose administered; remaining quantity; time; both signatures
- PRN CDS: assess for the target symptom before administration; contact the prescriber if any doubt about appropriateness
- Opioids specifically: assess respiratory rate before administration; do not administer if respiratory rate is below 10 breaths per minute without prescriber direction; Naloxone (Narcan) is accessible on site where opioids are in use
- Benzodiazepines: never abruptly stop; follow tapering schedule exactly; withdrawal from benzodiazepines can be life-threatening; any missed dose requires prescriber contact before the next dose

22. PSYCHOTROPIC AND PSYCHIATRIC MEDICATIONS

Psychotropic medications — antidepressants, antipsychotics, mood stabilizers, and anti-anxiety medications — are among the most commonly prescribed medications for youth in residential care and require careful monitoring and consistent administration.

Psychotropic Medication Category	Administration and Monitoring Requirements
Antidepressants (SSRIs, SNRIs: fluoxetine, sertraline, venlafaxine)	Consistency critical: must be taken at the same time daily; missed doses can cause discontinuation syndrome or mood instability; monitor for increased agitation, self-harm ideation, or suicidal ideation especially in the first 4–6 weeks or after dose changes; report any mood or behaviour change to PM and MCFD social worker
Antipsychotics (quetiapine, risperidone, aripiprazole, olanzapine)	Monitor for: sedation (especially the first week at any dose or dose change); extrapyramidal symptoms (restlessness, muscle stiffness, abnormal movements); metabolic changes (weight, blood sugar); document any of these and notify prescriber; some require fasting blood work — coordinate with prescriber
Mood stabilizers (lithium, valproate/Epival, lamotrigine/Lamictal)	Timing is critical — lithium especially requires consistent dosing; lithium toxicity is dangerous — monitor for: nausea and vomiting; tremor; confusion; irregular heartbeat; symptoms of toxicity are a medical emergency; valproate: monitor for bruising or unusual bleeding; lamotrigine: monitor for rash — any rash requires immediate prescriber notification
Stimulants for ADHD (methylphenidate, amphetamine salts)	Timing matters significantly: usually not given after 3 PM to avoid sleep disruption; document time precisely; monitor for: appetite suppression (note food intake at lunch); sleep; tics; cardiovascular effects; stimulants are CDS — double-lock and count apply
Anti-anxiety medications (non-CDS: hydroxyzine, buspirone)	Monitor for sedation; not typically PRN for youth unless specifically prescribed; document effectiveness

23. EMERGENCY MEDICATIONS AND AUTO-INJECTORS (EPIPEN)

23.1 Anaphylaxis and EpiPen

Where any youth has a documented anaphylaxis risk, a prescribed EpiPen (epinephrine auto-injector) is maintained at the program and accessible immediately. The EpiPen is a life-saving emergency medication — its use is never delayed.

Action	Procedure	Timeline
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Recognize anaphylaxis	Signs: hives; swelling of face, lips, tongue, or throat; difficulty breathing; wheezing; drop in blood pressure; vomiting; loss of consciousness; rapid onset after exposure to known allergen	Act immediately — anaphylaxis progresses rapidly
Administer EpiPen	Remove the EpiPen from its case; remove the safety cap (blue end); press the orange tip firmly against the outer thigh (can be through clothing); hold in place for 10 seconds; remove and massage the site for 10 seconds	Administer immediately on recognition of anaphylaxis — do not wait
Call 911	Call 911 immediately after or simultaneously with EpiPen administration; all anaphylaxis requires emergency medical assessment — even if the youth appears to improve; the EpiPen effect wears off and a second wave can occur	Call 911 immediately; do not delay to see if the EpiPen 'worked'
Second dose	If symptoms do not improve within 5–15 minutes and a second EpiPen is available: administer a second dose in the opposite thigh; await emergency services	After 5–15 minutes if no improvement
Position and monitor	Keep the youth lying down with legs elevated unless breathing is easier sitting; do not allow the youth to stand or walk; stay with the youth until emergency services arrive	Continuously until EMS arrives
Document and report	Document: the allergen exposure if known; time of EpiPen administration; dose and site; youth's response; time 911 was called; EMS response time; hospital destination	During and immediately after the event

23.2 EpiPen Maintenance

- EpiPen expiry date checked monthly on the facility inspection checklist; expired EpiPens replaced immediately
- EpiPen stored at room temperature (15–25°C); not refrigerated; not left in a hot vehicle
- EpiPen accessible immediately — not locked in the medication cabinet where it would take time to retrieve in an emergency; stored in a clearly labelled, accessible location known to all staff
- All staff complete EpiPen training (included in MALM); can demonstrate use on a training auto-injector
- After any EpiPen use: replace the auto-injector immediately; do not attempt to recap or reuse

23.3 Naloxone (Narcan)

Where any youth is prescribed opioid medications, a Naloxone kit is maintained at the program and all authorized staff are trained in its use.

- Signs of opioid overdose: unresponsive or extremely difficult to rouse; slow, shallow, or stopped breathing; blue lips or fingertips; pinpoint pupils
- Response: call 911 immediately; administer Naloxone per the kit instructions (nasal spray or injection); place the youth in the recovery position if unresponsive; rescue breathing if not breathing
- Naloxone effect wears off within 30–90 minutes — opioid effects may return; the youth must be assessed by emergency services regardless of apparent recovery
- BC Naloxone kits are available without prescription from most BC pharmacies; programs renew their kits at the program pharmacy

24. MEDICATIONS REQUIRING SPECIAL MONITORING

The following medication categories require specific monitoring that is documented in the shift log and the Individual Medication Profile. PC ensures monitoring instructions are current for each youth.

Medication	Monitoring Required	Action if Abnormal
Lithium	Weekly: observe for tremor, nausea, confusion, thirst. Requires regular blood levels — coordinate with prescriber for blood work appointments	Any signs of toxicity (nausea, tremor, confusion, irregular heartbeat): call the prescriber immediately; do not administer next dose without prescriber direction; 911 if severe
Valproate (Epival, Depakote)	Monthly: observe for unusual bruising or bleeding; watch for signs of liver toxicity (yellowing, severe nausea)	Any unusual bruising or signs of liver involvement: notify PC and prescriber same day
Lamotrigine (Lamictal)	Daily: observe for any rash anywhere on the body	Any rash: hold the dose; notify PC; call the prescriber immediately; any severe rash with fever requires 911
Antipsychotics	Weekly: observe for muscle stiffness, restlessness, abnormal movements; document weight monthly	Extrapyramidal symptoms: notify prescriber; do not stop medication abruptly without prescriber direction
Anticoagulants (warfarin)	Per prescriber: monitor for unusual bruising, bleeding from cuts that does not stop, blood in urine or stool	Any unusual bleeding: notify PC and prescriber same day
Seizure medications	Daily: observe for sedation; weekly: any signs of rash or unusual behaviour change	Breakthrough seizure: 911; document; notify prescriber;



(carbamazepine, levetiracetam)		do not adjust dose without prescriber direction
Diabetes medications (insulin, oral hypoglycemics)	Per prescriber protocol: blood glucose monitoring as directed; document all readings	Blood glucose outside target range per prescriber protocol: follow documented protocol; if hypoglycemic and symptomatic: follow emergency protocol

PART 6 — Medication Refusal and Non-Administration

25. YOUTH REFUSAL OF MEDICATION

A youth's refusal of medication is a clinical and rights issue, not a behaviour management situation. The youth has the right to refuse. Staff respond to refusal therapeutically and escalate according to the clinical significance of the medication being refused.

Step	Action	Documentation
1. Do not force	Medication is never administered by force. Never attempt to administer medication to a struggling, unwilling, or physically resisting youth. If physical force would be required: do not administer.	Document the refusal on the MAR (code R) and in the shift log
2. Do not coerce	Never threaten, bribe, manipulate, or use behaviour management leverage to pressure a youth to take medication. 'You won't get your phone back until you take your medication' is a prohibited practice.	Document that coercion was not used
3. Explore the reason	Calmly ask the youth why they don't want to take the medication. Common reasons: side effects; forgetting what it is for; wanting to feel in control; dislike of swallowing pills. Understanding the reason may allow a therapeutic response.	Document the youth's stated reason in the shift log
4. Provide information	Give the youth age-appropriate, accurate information about why the medication is prescribed and what it does. Support informed decision-making without coercion.	
5. Offer alternatives	If the medication has an acceptable alternative form (liquid vs tablet; different timing): consult the PC before offering an alternative; do not change the prescribed form without prescriber authorization.	
6. Allow time	If the medication is not immediately urgent, offer to check back in 15–30 minutes. Do not leave the medication unattended — return it to storage.	Document the second offer
7. Notify PC	Notify the PC of any medication refusal — same shift. PC assesses clinical significance and responds accordingly.	PC notified — time and name documented in shift log
8. PC escalation	PC contacts the prescriber if: the medication is critical (seizure medication, psychiatric	



	medication, antibiotic); refusal is recurring; the youth's condition may be affected. PC documents the prescriber contact.	
9. Complete documentation	Complete the Medication Refusal Record (Appendix H): youth's name; medication refused; reason given; steps taken; outcome; PC notified.	Medication Refusal Record completed before end of shift

! Medications are NEVER administered covertly — hidden in food, drink, or in any other disguised form — without explicit informed consent and prescriber authorization under exceptional clinical circumstances. Covert medication administration without authorization is a form of assault and a prohibited practice. Any instruction to administer medication covertly must be refused and reported to the PM and ED.

26. MISSED OR LATE DOSES

Situation	Response
Missed dose — youth unavailable (school, appointment, outing)	Check the Individual Medication Profile for instructions on missed doses for this medication; some medications: administer as soon as possible; some: skip and resume at next scheduled time; when in doubt: call the pharmacy. Document on MAR with explanation.
Late dose — within 30 minutes of scheduled time	Administer and document the actual administration time on the MAR. This is not a missed dose.
Late dose — more than 30 minutes past scheduled time	Consult the pharmacy or prescriber before administering. Some medications: administer now. Others: skip and wait for next scheduled dose. Never double-dose without prescriber direction. Document the consultation and the outcome.
Missed dose of critical medication (seizure, psychiatric, anticoagulant)	Call the prescriber or pharmacy immediately; do not make an independent decision about administering a late dose of a medication with a narrow therapeutic window or significant withdrawal risk; document the consultation and follow the prescriber's direction exactly
Youth away for an extended period (hospital, family visit)	Prepare a medication supply with written instructions and appropriate containers; ensure the receiving caregiver is briefed; for CDS: follow specific protocols for medication transfer; document all medication supplied and returned

27. MEDICATION WITHHELD BY CLINICAL DIRECTION

A medication may be withheld (held) by direction of the prescriber or in response to a specific clinical concern. This is distinct from a missed dose and distinct from a refusal.



- A medication is held only on explicit direction from the prescriber or pharmacist; a hold is documented with the prescriber's name, the instruction given, and the time it was received
- If a medication is held due to clinical signs (e.g., heart rate below threshold for a cardiac medication), the staff member must contact the prescriber for direction before withholding — withholding without direction is not authorized
- Held doses are documented on the MAR with code H; the shift log documents the hold instruction and the notification chain
- A held medication is never withheld again at the next dose without fresh prescriber direction unless the original hold instruction covered multiple doses

PART 7 — Medication Errors

28. DEFINITION AND CLASSIFICATION OF MEDICATION ERRORS

A medication error is any unintended act or omission in the medication process that has the potential to cause, or does cause, harm to a youth. Medication errors include: wrong medication, wrong dose, wrong route, wrong time, wrong youth, missed dose, documentation error, and near-misses. Near-misses — errors caught before reaching the youth — are reported and investigated with the same seriousness as actual errors, because they reveal the same system vulnerabilities.

Error Category	Examples	Severity
Wrong youth	Medication given to a youth for whom it was not prescribed	High — always report immediately; medical assessment required
Wrong medication	Incorrect medication retrieved from storage; look-alike/sound-alike error	High — always report immediately; medical assessment required
Wrong dose — underdose	Less medication than prescribed given	Moderate to High — report immediately; clinical assessment required
Wrong dose — overdose	More medication than prescribed given	High — report immediately; medical assessment required; 911 may be required
Wrong route	Medication administered by a route other than prescribed (e.g., oral medication given as ear drops)	High — report immediately; medical assessment required
Wrong time — significantly late or missed	Medication more than 30 minutes late; medication not administered and not documented; missed dose undocumented	Low to Moderate — report to PC same shift; clinical assessment for critical medications
Documentation error	MAR pre-signed; incorrect entry not corrected properly; entry not made; wrong time recorded	Moderate — report to PC same shift; correction required
Near-miss	Error caught before the medication reached the youth; Seven Rights prevented the error	Variable — report and investigate regardless of severity; near-misses are the most important errors to learn from

Adverse drug reaction (not an error but related)	Unexpected or harmful response to a medication administered correctly	Variable — see Section 36
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29. MEDICATION ERROR RESPONSE PROCEDURE

When a medication error occurs or is discovered, the immediate priority is the safety and wellbeing of the youth. Documentation and reporting follow immediately after.

Step	Action	Timeline
1. ASSESS THE YOUTH	Immediately assess the youth's condition. Is the youth experiencing any adverse effects? Altered consciousness? Difficulty breathing? Pain or distress?	Immediately
2. CALL 911 if indicated	Call 911 if: the youth is experiencing a serious adverse reaction; the error involved an overdose of any medication; the error involved a CDS; the youth is unresponsive or in distress.	Immediately if any clinical concern
3. Call Poison Control	For any error where you are uncertain about risk: call BC Poison Control at 1-800-567-8911 (24/7). Provide: youth's age and weight; the medication; the dose given; the dose prescribed; the time of administration. Follow their instructions exactly.	Immediately or concurrently with Step 1
4. Notify PC or on-call supervisor	Notify immediately — do not wait until end of shift. Provide all relevant information.	Within 15 minutes of discovering the error
5. Do not administer further doses	Do not administer the next scheduled dose until you have spoken with the PC and, where needed, the prescriber or pharmacist.	Immediately
6. Stay with the youth	Remain with the youth and monitor their condition until medical assessment is arranged or the PC arrives to assume oversight.	Continuously
7. PC notifies PM	PC notifies PM immediately after ensuring the youth is safe.	Within 30 minutes
8. PM notifies prescriber and pharmacist	PM contacts the prescriber and the dispensing pharmacy to report the error and receive direction on clinical management.	Within 1 hour
9. Document	Complete the Medication Error Report Form (Appendix D) before end of shift. Do not alter the	Before end of shift



	MAR; document the error accurately.	
10. PM notifies MCFD	PM notifies the MCFD social worker of the error. Errors involving overdose, wrong medication, wrong youth, or requiring medical intervention are Category 1 CIs.	Same day

BC Poison Control: 1-800-567-8911 (24 hours, 7 days). When calling: give youth's name and age; exact medication name; dose prescribed vs dose given; time administered; current symptoms. They will advise whether medical assessment is required and what to watch for. Follow their instructions exactly.

30. MEDICATION ERROR REPORTING AND DOCUMENTATION

30.1 MAR Correction Following an Error

The MAR is a legal document. An entry is never erased, obscured with correction fluid, or destroyed. Errors are corrected as follows:

- Draw a single line through the incorrect entry; do not obscure it
- Write the correct information above or beside the crossed-out entry
- Initial and date the correction
- If a wrong medication was administered: add a notation in the margin with the error details; never falsify the record to conceal an error
- The completed Medication Error Report (Appendix D) accompanies the corrected MAR in the youth's medication file

30.2 Medication Error Report Form (Appendix D)

A Medication Error Report is completed for every medication error and near-miss, without exception. Required information:

- Youth's name; medication involved; prescribed dose and route; actual dose and route; time administered; time error discovered
- Description of the error in the staff member's own words
- Contributing factors: what conditions led to this error?
- Immediate response taken: Poison Control called (yes/no; time); 911 called (yes/no; time); youth's condition
- Notification chain: PC name and time notified; PM name and time notified; prescriber notified; MCFD notified
- Staff member signature; PC signature; PM co-signature (within 24 hours)

31. ROOT CAUSE ANALYSIS AND PREVENTION



Every medication error and near-miss is reviewed by the PM within 5 business days using the following root cause analysis framework. The goal is system improvement, not individual blame.

Root Cause Category	Questions to Investigate
Staff factors	Was the staff member trained and authorized? Was this their first administration of this medication at this program? Were they fatigued or distracted? Was their MALM certification current?
System and process factors	Was the Seven Rights procedure followed? Was the MAR clear and up to date? Were the medications clearly labelled and stored correctly? Were look-alike/sound-alike medications stored together?
Communication factors	Was there a new prescription that had not been fully communicated to the administering staff? Was there a shift handover gap? Was the medication profile current?
Environmental factors	Was the medication administration environment free from interruptions? Was lighting adequate? Were there distractions?
Medication factors	Is this a high-alert medication? Is there a look-alike/sound-alike risk with other medications in the cabinet? Is the dosing complex?

Every root cause analysis generates at least one corrective action with a named owner and target date. Corrective actions are tracked in the Corrective Action Log and reported to the ED quarterly as part of the medication safety KPI report.

PART 8 — Medication Review and Disposal

32. MEDICATION REVIEW AND RECONCILIATION

32.1 Monthly Medication Review

The PC conducts a monthly medication review by the last business day of each month, covering:

- All medications in the cabinet counted and reconciled against the MAR; any discrepancy investigated and documented
- All medications checked for expiry; expired medications removed and disposed of immediately
- All medication labels verified for legibility and accuracy
- Medication storage conditions confirmed (temperature, light, security)
- All PRN medication records reviewed for frequency and effectiveness
- Any medications with upcoming prescription renewals flagged for PC follow-up
- Results documented on the Monthly Medication Review Checklist and submitted to PM within 5 business days

32.2 Prescriber Appointment Coordination

- PC maintains a medication renewal calendar for all youth
- Prescription renewals are initiated at least 2 weeks before expiry — never allow a prescription to lapse
- PC prepares a medication summary for every prescriber appointment: current medications; any concerns; refusals; PRN frequency and effectiveness; any observed side effects
- After every prescriber appointment: any medication changes are actioned immediately; MAR and medication profile updated before the next administration

33. EXPIRY, DISCONTINUATION, AND DISPOSAL

33.1 Expired Medications

- Expired medications are removed from circulation immediately upon discovery; they are never administered
- Expired medications are segregated from active medications; labelled 'EXPIRED — DO NOT USE'; secured until disposed of
- Disposal: BC Medications Return program at a participating pharmacy; never flushed down the toilet or placed in household garbage; never burned



33.2 Discontinued Medications

- When a medication is discontinued by the prescriber: remove from the active MAR immediately; update the Individual Medication Profile; segregate the remaining supply pending disposal
- Remaining supply of discontinued medications is disposed of via the BC Medications Return program
- CDS disposal: two authorized staff must witness the disposal; disposal is documented on the CDS Count Sheet and Medication Disposal Record (Appendix G) with both staff signatures; disposal via BC Medications Return program or as directed by the pharmacy and CDSA requirements

33.3 Disposal Records

All medication disposal is documented on the Medication Disposal Record (Appendix G): youth's name; medication name; quantity disposed; reason for disposal (expired, discontinued, returned); disposal method; date; two staff signatures for CDS.

34. TRANSFER AND DISCHARGE MEDICATION PROCEDURES

Transfer/Discharge Situation	Required Action
Youth transferred to another program or placement	PC prepares a complete Medication Transfer Package: current Individual Medication Profile; all active prescriptions (originals or certified copies); current MAR; remaining medication supply; CDS count documentation; written briefing for the receiving caregiver on any critical medication information; PC verbally briefs receiving PC or caregiver on any medication concerns
Youth admitted to hospital	Send all current medications with the youth; provide the emergency department with the complete medication list including allergies; notify the MCFD social worker; when the youth returns: reconcile any medication changes with the prescriber before resuming the prior regimen
Youth returning home — permanent discharge	Provide all remaining medications to the authorized caregiver with written instructions; provide the caregiver with the prescriber's contact information; document medications discharged; CDS returning with the youth require two-staff witnessed handover and documentation
Youth missing — medications left at program	Maintain all medications securely during the missing period; do not dispose of medications until the youth's status is resolved; if youth returns: reconcile any missed doses with the prescriber before resuming
Youth deceased	All medications are secured immediately; do not dispose of any medications; notify the PM and ED; await direction from MCFD, the prescriber, and the pharmacy; CDS may require police involvement per



	CDSA requirements
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PART 9 — Health Monitoring and Emergencies

35. MONITORING MEDICATION EFFECTS AND SIDE EFFECTS

Medication monitoring is a continuous responsibility of all staff — not only at the time of administration. Staff observe for medication effects and side effects throughout every shift and document relevant observations in the shift log.

35.1 Shift Monitoring Expectations

- Know each youth's current medications and their expected effects and common side effects; this information is in the Individual Medication Profile and the pharmacy medication information sheet
- Document relevant observations in the shift log: appetite and food intake; sleep; mood and behaviour; energy level; any new physical complaints; any observed side effects
- For medications with specific monitoring requirements (Section 24): conduct and document the required monitoring at the specified intervals
- Report any concerning observation to the PC or on-call supervisor immediately — do not wait until the next shift to report a medication concern

35.2 Communication with Prescribers

- The PC maintains a log of medication concerns to raise at each prescriber appointment; any urgent concern is communicated to the prescriber on the same day
- Staff observations in the shift log are the primary source of medication effectiveness and side effect data for prescriber decision-making; thorough documentation directly supports better clinical care

36. ADVERSE DRUG REACTIONS

An adverse drug reaction (ADR) is an unintended, harmful response to a medication administered at the correct dose. ADRs range from mild (rash, nausea) to severe (anaphylaxis, liver failure). An ADR is not a medication error, but it requires the same urgency in response.

ADR Severity	Signs	Response
Mild	Mild nausea; loose stool; mild rash; mild drowsiness; mild headache	Document in shift log; notify PC; PC notifies prescriber at next appointment or same day if concerning
Moderate	Significant nausea/vomiting; persistent rash; significant drowsiness affecting function;	Notify PC immediately; PC contacts prescriber same



	unusual movements; moderate allergic symptoms	day; do not administer next dose without prescriber direction; document fully
Severe — potential emergency	Difficulty breathing; throat swelling; severe rash (especially with fever — Stevens-Johnson syndrome risk); irregular heartbeat; loss of consciousness; seizure; severe allergic reaction	Call 911 immediately; do not administer next dose; notify PC and PM; complete ADR documentation; MCFD notification if hospitalization results

37. MEDICAL EMERGENCIES RELATED TO MEDICATION

Emergency	Response
Suspected overdose — accidental or intentional	Call 911 immediately; call Poison Control 1-800-567-8911 simultaneously or immediately after; do not induce vomiting unless specifically directed by Poison Control; bring the medication container to the emergency department; stay with the youth; notify PC and PM; MCFD notification same day
Anaphylaxis — severe allergic reaction	Administer EpiPen immediately if prescribed; call 911 immediately; follow the EpiPen protocol (Section 23.1); do not leave the youth
Seizure following medication change	Call 911; do not restrain the youth; clear the area of hazards; time the seizure; stay with the youth; notify PC and PM; notify prescriber as soon as 911 has been called
Severe sedation or unresponsiveness	Do not leave the youth; call 911; if opioids involved: administer Naloxone if available and trained; keep airway open; recovery position if breathing; rescue breathing if not breathing; notify PC and PM
Medication ingestion by wrong youth (different youth accessed another youth's medications)	Treat as a medication error: assess both youth; call Poison Control; call 911 if any clinical concern; notify PC and PM immediately; MCFD notification for both youth; full incident report



PART 10 — Quality Assurance and Oversight

38. MAR AUDIT AND QUALITY REVIEW

38.1 Monthly MAR Audit

The PC conducts a monthly MAR audit for every youth with active medications by the 10th of the following month. The audit reviews the prior month's MAR for:

Audit Item	Standard
Completeness	Every scheduled dose has an entry; no blank spaces; all entries initialled
Refusal documentation	All refusals have a code R; a shift log entry and Medication Refusal Record exist for each refusal
Held doses	All held doses have a code H; prescriber instruction documented in the shift log
PRN documentation	All PRN administrations have a corresponding PRN record (Appendix F)
CDS counts	CDS Count Sheet reconciles with the MAR for the entire month; all counts signed by two staff
Error correction	Any corrections made using correct procedure (single line, initialled, dated)
No pre-signing	No evidence of pre-signed entries (entries made before the medication was administered)
Signature legend	All initials expanded in the signature legend
Expiry dates	No medications administered past their expiry date

PC documents the audit findings; any anomalies are investigated with the involved staff; PM reviews the monthly audit summary by the 15th of the month.

39. QA INDICATORS — MEDICATION SAFETY

Indicator	Target	Reporting Frequency
Medication errors — total (including near-misses)	Decreasing year-over-year; zero preferred	Monthly PC report; quarterly PM trend to ED



Medication errors requiring medical assessment	Zero — zero tolerance for preventable serious errors	Monthly; immediate PM and MCFD notification
CDS count discrepancies — unresolved	Zero	Monthly; immediate PM notification
Staff with current medication administration authorization	100%	Monthly by 5th; PC confirms
MARs with pre-signed entries identified	Zero	Monthly audit
Medication refusals — number and pattern	Tracked; recurring refusals trigger prescriber review	Monthly; recurring refusals flagged to PM
PRN medications used more than 3 times in 7 days	Flagged for prescriber review	Weekly PC review of PRN records
Expired medications found in active supply	Zero — monthly inventory prevents	Monthly inventory audit
EpiPen current and accessible (where applicable)	100%; checked monthly	Monthly inspection checklist
Medication training compliance — all authorized staff	100% MALM current	Monthly; PC confirms by 5th

40. PHARMACY AND PRESCRIBER RELATIONSHIPS

- Each program maintains a primary pharmacy relationship; pharmacy name and contact numbers are posted in the medication storage area and in this manual
- The PC contacts the pharmacy with any medication question — before administering any medication about which there is doubt
- The pharmacy is the first point of contact for: drug interaction questions; storage questions; expiry and disposal questions; label concerns; dose calculation verification
- The PM maintains the prescriber contact list for all youth; prescribers are accessible for urgent clinical questions
- Blister pack dispensing is preferred for all youth with multiple medications; the PC requests blister pack dispensing from the pharmacy at each prescription renewal
- Any pharmacy error (wrong medication dispensed, wrong dose labelled) is reported to the pharmacy immediately; a Medication Error Report is completed; the error is reported to the college or licensing authority if required



PART 11 — Appendices



APPENDIX Appendix A — STAFF MEDICATION ADMINISTRATION AUTHORIZATION FORM

Completed by the Program Manager for every staff member authorized to administer medications. Copy filed in: staff personnel file; program medication authorization binder. Authorization is program-specific and expires 12 months from issue.

Staff Member Name (print)	
Position	
Program Name and Address	
Training Completed — BC MALM or equivalent (course name, date, certificate number)	
Competency Demonstration Date	
Competency Assessed By (PC or PM name)	
Medications Authorized to Administer	All scheduled and OTC medications per current Individual Medication Profiles / All EXCEPT: _____
CDS Authorization (yes / no)	
EpiPen Administration Authorization (yes / no)	
Naloxone Administration Authorization (yes / no)	
Authorization Effective Date	
Authorization Expiry Date (12 months from issue)	
Conditions or Limitations on Authorization	
Program Manager Signature	
Program Manager Name (print)	
Date	
Annual Renewal — Date; PM Signature	







APPENDIX Appendix B — MEDICATION ADMINISTRATION RECORD (MAR) TEMPLATE

One MAR per youth per calendar month. One line per medication. Complete all identification fields before any entries. Entry codes: initials = administered; R = refused; H = held; NA = not available; S = self-administered. All initials expanded in signature legend below.

Youth Legal Name	
Date of Birth	
Program Name	
Month / Year	
Known Allergies — prominently displayed	
Program Coordinator	
Program Manager	

Medication Name; Dose; Route	Prescriber	Scheduled Time(s)	Day 1 — Day 31 (enter code in each cell)
		AM:	
		Noon:	
		PM:	
		HS:	
		PRN:	
		Other:	

SIGNATURE LEGEND — All administering staff expand initials here	Full Name / Signature / Initials



PC Monthly Review — Signature and Date	
PM Review — Signature and Date	





APPENDIX Appendix C — CONTROLLED DRUG AND SUBSTANCE COUNT SHEET

Completed at every shift change by two authorized staff. One row per administration. Running balance maintained. Both staff sign every entry. Any discrepancy investigated before the outgoing shift departs.

Youth Legal Name	
Medication Name; Strength; Form	
Program Name	
Month / Year	
Opening Count (beginning of month)	

Date / Time	Dose Administered (or count only)	Balance Remaining	Administered by / Witnessed by
	Shift change count only — no dose		

Month-End Count Reconciliation	
Opening count	
Plus: received during month	
Minus: administered during month (per MAR)	



Minus: disposed during month	
Expected closing count	
Actual closing count	
Discrepancy (if any)	
Explanation of discrepancy	
PC Signature	
PM Signature	



**APPENDIX Appendix D — MEDICATION ERROR REPORT FORM**

! Complete this form for every medication error and near-miss. Do not alter or destroy. PM co-signs within 24 hours. This is a legal document.

Youth Legal Name	
Date and Time of Error	
Date and Time Error Discovered	
Name of Staff Member Who Made the Error	
Name of Staff Member Who Discovered the Error	
Medication Involved — name; prescribed dose; prescribed route	
What Was Actually Administered — medication; dose; route	
Error Category (circle)	Wrong youth / Wrong medication / Wrong dose / Wrong route / Wrong time / Documentation / Near-miss / Other:
Describe the error in detail (what happened, step by step)	
Contributing factors (what conditions led to this error?)	
Was this a near-miss? (error caught before reaching the youth)	Yes / No
Youth's condition at the time the error was discovered	
Poison Control called? (1-800-567-8911)	Yes / No / Not required Time called: _____ Advice received:
911 called?	Yes / No If yes, time called: _____ EMS response and outcome:
Prescriber / pharmacy notified?	Yes / No Name: _____ Time: _____ Instructions received:



PC notified — name and time	
PM notified — name and time	
MCFD social worker notified — name and time	
Youth's condition at end of shift	
Immediate corrective action taken	
Recommended system change to prevent recurrence	
Staff member signature and date	
PC signature and date	
PM co-signature and date (within 24 hours)	
Root cause analysis completed by (PM) — date	





APPENDIX Appendix E — MEDICATION INCIDENT — POISON CONTROL AND EMERGENCY REFERENCE

Post this card in the medication storage area and in the staff communication area. All authorized staff know these numbers.

BC Poison Control (24/7)	1-800-567-8911
911 — all medication emergencies involving altered consciousness, breathing difficulty, anaphylaxis, or overdose	911
On-Call Supervisor	[See posted weekly on-call schedule]
Program Pharmacy — name and number	[Insert program pharmacy name and number]
On-Call Prescriber (after hours)	[Insert prescriber or clinic after-hours number]
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122

WHEN TO CALL 911 — MEDICATION-RELATED EMERGENCIES	
Youth is unresponsive or very difficult to rouse	Call 911 immediately
Youth is having difficulty breathing	Call 911 immediately
Youth is having a seizure and has no seizure history, or the seizure lasts more than 5 minutes	Call 911 immediately
Suspected medication overdose — intentional or accidental	Call 911 immediately; also call Poison Control
Signs of anaphylaxis: throat swelling, hives, breathing difficulty, drop in blood pressure	Administer EpiPen if prescribed; call 911 immediately
Any severe adverse drug	Call 911; call Poison Control



reaction	
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WHEN TO CALL POISON CONTROL — 1-800-567-8911	
Have ready when you call	Youth's age and weight; medication name; prescribed dose; actual dose given; time administered; current signs and symptoms
Any suspected overdose — accidental	Call Poison Control and follow their instructions exactly
Any unexpected reaction to a medication	Call Poison Control if uncertain whether 911 is required
Medication ingested by the wrong youth	Call Poison Control for both medications; call 911 if any concern
Uncertainty about the risk of any medication error	Call Poison Control — they will advise whether monitoring at home or emergency assessment is required





APPENDIX Appendix F — PRN MEDICATION ADMINISTRATION AND EFFECTIVENESS RECORD

Complete for every PRN administration. PC reviews weekly. If PRN used more than 3 times in 7 days: notify PM; PC contacts prescriber.

Youth Legal Name	
Medication Name; Dose; Route	
PRN Indication — target symptom or condition	
Maximum Dose per Administration	
Maximum Frequency (e.g., every 4 hours; maximum 3 times per day)	
Prescriber	
PRN Protocol Approved By (PM)	

Date / Time	Pre-Administration Assessment (symptom present; severity)	Dose Given	Post-Administration Assessment (effectiveness; onset; side effects)

PC Weekly Review — Signature and Date	
7-Day Usage Count	



Prescriber Notified? (if 3+ uses in 7 days)	Yes / No Date: _____ Outcome:
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APPENDIX Appendix G — MEDICATION DISPOSAL RECORD

Complete for every medication disposal. CDS disposals require two-staff signatures. Retain with the medication file for 7 years (10 years for CDS).

Date	Medication Name; Strength	Youth Name	Quantity Disposed

Reason for Disposal	Disposal Method	Staff Signature(s) — Two Required for CDS
Expired / Discontinued / Returned at discharge	BC Medications Return Program at: _____	



**APPENDIX Appendix H — YOUTH MEDICATION REFUSAL RECORD**

Youth Legal Name	
Date and Time of Refusal	
Medication Refused — name; dose	
Youth's Stated Reason for Refusal	
Staff Response — steps taken	
Was a second offer made? (yes / no; time of second offer)	
Outcome — medication eventually taken; refused; held	
PC Notified — name and time	
Was prescriber contacted? (yes / no; if yes, prescriber name and advice)	
Was medication administered covertly at any point? (must be 'No')	No
Clinical significance assessed?	
MCFD notification required?	
Staff Member Signature	
Date	
PC Co-Signature	





APPENDIX Appendix I — MEDICATION ADMINISTRATION STAFF ACKNOWLEDGEMENT FORM

All staff authorized to administer medications must read and sign this acknowledgement before their first medication administration duty and upon each annual update of this manual.

I acknowledge that I have received, read, and understood the Evergreen Community Care Medication Administration Manual (ECC-MED-2026-001), Version 1.0, effective July 1, 2026.	
I understand that I may not administer any medication until I hold current written program-specific authorization from the Program Manager.	
I understand that every medication administration requires verification of the Seven Rights before administering any dose.	
I understand that pre-signing the MAR before administering a medication is absolutely prohibited.	
I understand that medications are never administered by force, covertly, or as a behaviour management tool under any circumstances.	
I understand that every medication error and near-miss must be reported immediately to the PC or on-call supervisor, and documented on the Medication Error Report Form (Appendix D) before the end of the shift.	
I understand that controlled drugs and substances require two-staff counts at	



every shift change, double-locked storage, and that any discrepancy is investigated before the outgoing shift departs.	
I understand that BC Poison Control (1-800-567-8911) is called for any medication error or unexpected reaction where I am uncertain about the risk to the youth.	
I understand that my medication administration authorization is program-specific and expires 12 months from its issue date.	
I understand that questions about medications are directed to the PC, PM, the pharmacy, or the prescriber — never guessed.	

Staff Name (print)	
Position	
Program	
Signature	
Date	
Current MALM Certificate Expiry Date	
Current Medication Authorization Expiry Date	
PC Signature (confirms authorization current and on file)	
Date	



Version History

Version	Date	Summary
1.0	July 1, 2026	Initial standalone Medication Administration Manual for Evergreen Community Care; 40 sections in 11 Parts; 9 Appendices (Staff Authorization Form, MAR Template, CDS Count Sheet, Medication Error Report, Poison Control Reference, PRN Record, Disposal Record, Refusal Record, Staff Acknowledgement); aligned to RCR s.45-49; CDSA; BC MALM; MCFD SHSS Standard G.3 and D.3; EpiPen and Naloxone emergency procedures; covert medication prohibition; Seven Rights framework; root cause analysis framework for errors

Signatory	Signature / Date
Executive Director	Signature: _____ Date: _____